

Surgery for pancreatic cancer

Find out what to expect before and after your surgery for pancreatic cancer.

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Having surgery for pancreatic cancer

Your surgeon and specialist nurse will explain your operation and what to expect before and after surgery.

We have more information about [different types of surgery](#) for pancreatic cancer.

Some people have [chemotherapy](#) before surgery to shrink the cancer. We also have information about surgery and other treatments to [control the symptoms of pancreatic cancer](#).

Before your operation

If you smoke, try to give up or smoke less before your operation. This will lower your risk of problems, such as a chest infection. It will also help your wound to heal after the operation. Your GP can give you advice. You may also find it helpful to read our information on [giving up smoking](#).

[Before surgery](#), you will have an appointment at the pre-assessment clinic. At the clinic, you will have tests to check you are fit for the operation. These may include [blood tests](#), a [chest x-ray](#) or a recording of your heart (ECG). You will meet a member of the surgical team to discuss the operation. This is a good time to share any questions or concerns you have.

You may need help when you go home after surgery if you live alone or care for someone else. You should tell your nurse as soon as possible. This will give them plenty of time to make arrangements for anything you need to go home.

Some hospitals follow an enhanced recovery programme. This aims to reduce your time in hospital and to speed up your recovery. It also helps you become more involved in your own care. For example, you may get information about diet and exercise. The team will also make any arrangements you need to go home. Your doctor will tell you if an enhanced recovery programme is available and suitable for you.

You will usually be admitted to hospital on the morning of the operation. You will be given elastic stockings (TED stockings) to wear during the operation and for some time afterwards. This is to reduce the risk of blood clots in your legs.

After surgery

Knowing what will happen when you wake up after your operation can help you feel less anxious. It also prepares your family and friends for what to expect. Your recovery time will depend on the [type of surgery](#) you have.

Most people receive care in a surgical high-dependency unit for a few days after their operation. This is routine. Some people are cared for in the intensive care unit (ICU) if the operation was longer or more complicated.

You will probably feel quite drowsy and may not remember much about the first days after your operation.

The nurses and physiotherapists will encourage you to start moving about as soon as possible. This is an important part of your recovery. It will help prevent problems such as chest infections or blood clots. You will be encouraged to do regular leg movements and deep breathing exercises. A physiotherapist or a nurse can explain these to you.

Drips and drains

For a short time after the operation, you may have some of the following:

- A drip going into a vein in your arm or neck - this is called an intravenous infusion. This gives you fluids until you can eat and drink again. It can also give you painkillers and other medications.
- One or more drainage tubes coming from your wound. These drain away fluids, such as blood, bile or pancreatic juices. You will have these drainage tubes until the amount of fluid draining away has reduced.
- A fine tube that passes down your nose and into your stomach or small intestine. This is called a [nasogastric tube](#). It drains fluids from your stomach so that you do not feel sick. You may need this for several days.
- A small, flexible tube called a catheter in your bladder. This will drain into a collecting bag. This means you will not have to get up to pass urine. You will usually only have this for a couple of days.

Pain control

It is normal to have some pain and discomfort after your operation. This can be controlled with [painkillers](#). If you are in pain, tell the nurses. They may need to change the dose or type of painkiller.

You may have a spinal block during the operation. This is an injection of long-lasting painkiller into the fluid around the spinal cord. It gives pain relief for up to 24 hours. Or you may have an epidural. This is a pump that continuously delivers painkiller into the spinal fluid through a fine tube and can be left in for up to 5 days after the operation.

Sometimes a small, plastic tube is placed into the muscle around the wound, so that painkillers can be given through it for a couple of days.

Painkillers can also be given through a tube into a vein in your hand or arm. This is called a cannula. The tube connects to a pump. This is called PCA (patient-controlled analgesia). You can give yourself an extra dose of painkiller when you need it by pressing a button. The machine is set so you get a safe dose and cannot have too much.

Before you go home, your pain will be controlled by tablets or patches. You will get a prescription for painkillers you can take at home as needed.

Eating and drinking

You will not be able to eat or drink straight after the operation. Until you can, you will have fluids through a drip. Some people have extra nutrition through the drip or a feeding tube.

Depending on the operation you had, you may be able to start drinking small amounts of fluid the day after the operation. The surgeon will tell you when you can. You may have supplement drinks for a few days. These will give you the nutrition you need to help your recovery.

If the operation you had means you will not be able to eat and drink for a few days, you will be given nutrition through a feeding tube. You may have a feeding tube called a [nasogastric tube \(NG tube\)](#) or a [nasojejunal tube \(NJ tube\)](#). An NG tube is passed through the nose and into the stomach. An NJ tube is passed through the nose and stomach, into the small bowel. It is used if you have a [Whipple's procedure](#).

Wound care

The surgeon will close the wound with stitches, clips, or staples. These are usually removed 10 to 14 days after the operation. If you have gone home, your practice nurse can do this at your GP surgery. If you cannot leave home, a district nurse can visit you. Sometimes stitches are under the skin. These do not need to be removed as they dissolve on their own.

You may have antibiotics to help prevent wound infection. You will have a dressing covering your wound. This may be left in place for the first few days.

It is important to let your nurse or doctor know straight away about any problems with your wound. Tell them if the wound becomes hot or painful, or if it begins to bleed or leak fluids, even after going home.

Going home

Depending on the type of operation and your recovery, you can probably go home 5 to 10 days after surgery. Your doctor or specialist nurse will be able to give you more information.

You will have an appointment at an outpatient clinic for your post-operative check-up. This lets you discuss any further treatment you may need with them, such as [radiotherapy](#) or [chemotherapy](#).

Replacing insulin and digestive juices

Insulin injections

Sometimes, if you have had part of your pancreas removed, your body may struggle to control blood sugar. This is because the remaining pancreas may not be making enough insulin. You may become diabetic and need to have insulin injections or tablets to control your blood sugar levels.

If you have had an operation to remove your whole pancreas, you will need daily insulin injections. You will need to have these injections for life. These are given under the skin (subcutaneously). You will also need to closely check your blood sugar level regularly. A diabetes nurse will give you support and advice about insulin replacement. If you already have diabetes, you may have to increase your insulin or tablet dose.

Enzyme replacement

You may need to replace the digestive enzymes that the pancreas normally makes. This will depend on the type of operation you had, whether you still have part of your pancreas and how well the remaining pancreas is working. Your surgeon, nurse or dietitian will give you more information.

Digestive enzymes help your body to break down and absorb fats and protein. If you do not have enough of them, you may have diarrhoea. Your stools (poo) may float, smell bad and be difficult to flush. This is called steatorrhoea. You will also find it difficult to put on weight, because you cannot absorb nutrients from your food.

You can replace the digestive enzymes by swallowing capsules. You will usually take these capsules with every meal and snack. Your nurse or a dietitian will explain how to take the capsules. You will need to take them for the rest of your life.

Commonly used pancreatic enzyme replacement therapy drugs are:

- Creon®
- Nutrizym®
- Pancrease®
- Pancrex®.

Most replacement enzymes are made from pork. Vegetarian enzymes are not available.

The most commonly used supplement is a tablet called Creon, which is a pork-based enzyme replacement. There is no alternative that is not pork-based. It has been approved for use by Jewish patients by the Chief Rabbi. Previous rulings by Islamic scholars suggest that Muslims may use pork-based medicines if there is no alternative. If you have concerns about this, speak to your religious leader.

About our information

References



Below is a sample of the sources used in our pancreatic cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

European Society for Medical Oncology, Cancer of the pancreas: ESMO clinical Practice Guidelines. Volume 26, Supplement 5, V56-V68, 1 September 2015. Available from: <https://doi.org/10.1093/annonc/mdv295> (accessed May 2021).

National Institute for Health and Care Excellence. Pancreatic cancer in adults: diagnosis and management. NICE guideline (NG85). Published 7 February 2018. Available at: <https://www.nice.org.uk/guidance/ng85/chapter/Recommendations> (accessed May 2021).

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Chief Medical Editor, [Professor Tim Iveson](#), Consultant Medical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

Date reviewed

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⚠ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help